

Predominant comedonal acne

Topical treatment with either a retinoid, azelaic acid, or salicylic acid is recommended as a first-line approach for patients with predominant comedonal facial acne without significant inflammatory lesions. Topical retinoids are considered more effective than azelaic acid, which in turn is more effective than salicylic acid. Topical retinoids and azelaic acid inhibit the formation of microcomedones—the precursor to all acne lesions—and are both comedolytic and anti-comedogenic, with additional anti-inflammatory properties. Retinoid-based therapy is recommended as a first-line treatment for almost all acne patients by the Global Alliance.

The dose or frequency of application of first-line treatments may be reduced if patients experience adverse events such as skin irritation.

Maintenance therapy

Maintenance therapy with a topical retinoid or azelaic acid is recommended once a patient has responded to treatment and is clear or nearly clear of acne. The goal is to prevent recurrence by suppressing microcomedone formation, which can persist in clinically normal-appearing skin. Retinoids are generally preferred over azelaic acid due to stronger evidence of efficacy, although recent data suggest that azelaic acid and adapalene have comparable effectiveness in adult female acne.

Dose or frequency should be reduced if irritation occurs. Salicylic acid at concentrations $>5\%$ for over 12 weeks or 10% for over 6 weeks is not recommended for maintenance due to potential systemic toxicity. Benzoyl peroxide (BPO) monotherapy should not be used for maintenance because it does not effectively target microcomedones or comedones. Topical antibiotic monotherapy is also not recommended, as long-term use may promote antibiotic-resistant *Cutibacterium acnes*.

Predominant papulopustular acne

For patients with predominant papulopustular acne, disease severity should be classified as:

- **Mild:** Grades 1–2
- **Moderate:** Grade 3
- **Severe:** Grades 4–5

Mild papulopustular acne (grades 1–2)

First-line topical treatments include: - Benzoyl peroxide (BPO) - Second-generation retinoids (e.g., adapalene) - Azelaic acid

Alternatively, fixed-dose combination therapies may be used, such as: - Clindamycin 1% / tretinoin 0.025% - Adapalene 0.1% / BPO 2.5% - Clindamycin 1% / BPO 3% or 5%

The choice of agent should be individualized based on patient characteristics, tolerability, and preferences.

Moderate papulopustular acne (grade 3 to 4)

Topical fixed-dose combination therapies are recommended as the preferred first-line approach.